

FAX COVER

To: Dr. Olivia Marsh (GP) | From: Hematology Ward | Date: 03.04.2024

Re: **Gary Baker** (AML036, DOB 01.12.1947)—AML **NPM1-mut/FLT3-ITD+**, on **7+3 + midostaurin** (ongoing)

HOSPITAL PROGRESS NOTE (abbrev.)

Dx: AML, ELN 2022 **Intermediate** (NPM1-mutated with FLT3-ITD) on **19.05.2023**.

Tx: Induction 7+3 started **26.05.2023**; **midostaurin 50 mg PO bid D8–21** each cycle. Achieved CR post-induction. Currently receiving **consolidation HiDAC ×3** with midostaurin continuation.

Interim Hx: Doing well at home between cycles. No recent fevers. Minimal nausea responsive to ondansetron. Using antibacterial and antiviral prophylaxis during nadirs; azole avoided to prevent CYP3A interaction with midostaurin.

Vitals/Exam: BP 128/72, HR 76, afebrile; clear lungs; no mucositis; abdomen benign; no petechiae. ECOG 1.

Latest Labs (02.04.2024): WBC 4.2 (ANC 1.8), Hgb 11.9 g/dL, Plt 186; Cr 0.98, AST/ALT 28/24; QTc 452 ms on ECG (stable). LDH normal.

Path/MRD:

- Day-28 marrow (June 2023) CR with blasts <2%; **NPM1 MRD by qPCR low-positive** initially (2.5×10^{-4}) → **converted to undetectable** after consolidation #2.
- FLT3-ITD not detected on post-therapy NGS.

Comorbidities/Medications:

- CAD s/p stent 2017 (on **atorvastatin 40 mg qHS**, **metoprolol succinate 50 mg qd**; **aspirin held** during thrombocytopenia, otherwise 81 mg qd).
- BPH (tamsulosin 0.4 mg qHS).
- GERD (pantoprazole 40 mg qAM).

Plan:

1. Proceed with consolidation #3 + midostaurin; growth-factor support post-cycle.
2. **Transplant eval:** HLA typing completed; proceed to consult only if MRD re-emerges or relapse biology changes—current MRD negative, so observation after consolidation is reasonable.

3. Continue infection prophylaxis during ANC <0.5 (levofloxacin/acyclovir).
4. Avoid strong CYP3A inhibitors (azole antifungals, certain macrolides).
5. Follow-up CBC twice weekly during nadir; clinic in 2–3 weeks.

Summary: Mr. Baker maintains a hematologic **CR** on **7+3 + midostaurin** with MRD clearance during consolidation. We will complete planned therapy and then transition to surveillance if counts and MRD remain favorable.

Signed, **[Name], PA-C** / Attending **[Name], MD**